

[Cancer of the prostate. Should men be screened, how to screen, when to screen?].

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The individual screening of the general population can only be envisaged after the age of 50 years when the incidence of prostatic cancer increases significantly. Individual screening by PSA in an asymptomatic subject must only be proposed to men likely to benefit from curative treatment, i.e. between the ages of 50 and 70 years, and only after performing digital rectal examination. The combination of these two methods detects 54% of prostatic cancers in men over the age of 50 years. PSA assay can be made more reliable by comparison with age-dependent normal limits, PSA density compared to prostatic volume, rate of progression, and assay of the free and bound fractions.

Prostate cancer screening: more harm than good?

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The American Cancer Society and other national medical organizations emphasize the need for routine screening for prostate cancer in men over the age of 50. The serum prostate-specific antigen (PSA) assay is the test most commonly recommended for the purpose of screening. However, when PSA screening is examined critically from the standpoint of the principles of screening, evidence from prospective studies to support the routine use of PSA testing is lacking. Data suggest that screening often detects what may be indolent, nonaggressive prostate cancer. The treatment of such a cancer with radiation or radical prostatectomy can result in significant morbidity, including urinary incontinence and impotence, without a proven decrease in mortality. Evidence from randomized clinical trials in support of routine PSA screening is urgently needed.

Screening for prostate cancer: a guidance statement from the Clinical Guidelines Committee of the American College of Physicians.

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Prostate cancer is an important health problem in men. It rarely causes death in men younger than 50 years; most deaths associated with it occur in men older than 75 years. The benefits of screening with the prostate-specific antigen (PSA) test are outweighed by the harms for most men. Prostate cancer never becomes clinically significant in a patient's lifetime in a considerable proportion of men with prostate cancer detected with the PSA test. They will receive no benefit and are subject to substantial harms from the treatment of prostate cancer. The American College of Physicians (ACP) developed this guidance statement for clinicians by assessing current prostate cancer screening guidelines developed by other organizations. ACP believes that it is more valuable to provide clinicians with a rigorous review of available guidelines rather than develop a new guideline on the same topic when several guidelines are available on a topic or when existing guidelines conflict. The purpose of this guidance statement is to critically review available guidelines to help guide internists and other clinicians in making decisions about screening for prostate cancer. The target patient population for this guidance statement is all adult men. This guidance statement is derived from an appraisal of available guidelines on screening for prostate cancer. Authors searched the National Guideline Clearinghouse to identify prostate cancer screening guidelines in the United States and selected 4 developed by the American College of

Preventive Medicine, American Cancer Society, American Urological Association, and U.S. Preventive Services Task Force. The AGREE II (Appraisal of Guidelines, Research and Evaluation in Europe) instrument was used to evaluate the guidelines. ACP recommends that clinicians inform men between the age of 50 and 69 years about the limited potential benefits and substantial harms of screening for prostate cancer. ACP recommends that clinicians base the decision to screen for prostate cancer using the prostate-specific antigen test on the risk for prostate cancer, a discussion of the benefits and harms of screening, the patient's general health and life expectancy, and patient preferences. ACP recommends that clinicians should not screen for prostate cancer using the prostate-specific antigen test in patients who do not express a clear preference for screening. GUIDANCE STATEMENT 2: ACP recommends that clinicians should not screen for prostate cancer using the prostate-specific antigen test in average-risk men under the age of 50 years, men over the age of 69 years, or men with a life expectancy of less than 10 to 15 years.

Brief report: Free prostate-specific antigen test utilization. Consistency with guidelines.

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The American Cancer Society and American Urologic Association recommend prostate cancer screening for average-risk men between the ages of 50 and 75 years using digital rectal examination and prostate-specific antigen (PSA) testing. Measuring the percent free PSA may improve test specificity for detecting prostate cancer when the total PSA is between 2.5 and 10 ng/mL. To assess whether free PSA testing practices are consistent with published screening guidelines. Retrospective analysis of free PSA testing performed by a national reference laboratory between October 1, 2003 and September 30, 2004. Free PSA and total PSA results and the age of the patient at testing. Over 24% of free PSA tests were performed on patients greater than 75 years of age, and 38% were performed on patients with a total PSA either less than 2.5 ng/mL or greater than 10.0 ng/mL. A substantial proportion of free PSA tests performed in a national reference laboratory appeared to be inconsistent with existing screening guidelines. This raises concern that some of these patients may have received inappropriate diagnostic workup and/or therapy, resulting in excess medical costs and potential harms.

Prostate-specific antigen in screening of prostate cancer.

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Prostate-specific antigen (PSA) is a well-characterized human prostate-specific glycoprotein. PSA has been shown to be the most effective immunohistologic marker for prostate cancer, as well as the most useful serologic test in staging and monitoring prostate cancer and in early detection of recurrent disease. The greatest clinical value of PSA is an aid for early detection of prostate cancer. Recent studies have indicated that PSA-based screening of the older population for organ-confined early-stage prostate cancer is an acceptable, practical, and reliable modality. The accuracy of PSA screening is within the same range as the mammogram. The cost-effectiveness of PSA is comparable to other cancer screening tests. Although the increase in the patient's survival due to PSA-based detection of early prostate cancer remains to be documented, it is generally agreed that the PSA test along with digital rectal examination (DRE) should be included in the annual physical examination for men 50 years of age or older. High-risk men are urged to commence at age 40. Asymptomatic men who have both a negative DRE and normal PSA blood test need only to continue an annual DRE and PSA check-up. Men who have a negative DRE

and elevated PSA, and all those who have a suspicious DRE regardless of PSA results, should undergo further diagnostic workup, such as transrectal ultrasonography with biopsy of visible lesions. The cure rate is high with timely treatment, when prostate cancer is detected while still confined to the prostate.

Prostate cancer screening: Attitudes and practices of family physicians in Ontario.

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: The utility of prostate cancer screening is controversial. We sought to determine whether Ontario's family physicians believe it is beneficial and to characterize their screening protocols. : A survey was developed with input from urologists, family physicians and the Ontario Medical Association's Section on General and Family Practice. Questions covered three domains: (1) demographics, (2) beliefs about screening utility and (3) screening practices. All 7302 family physicians in Ontario were invited by email to complete the online survey. : A total of 969 physicians completed the survey; 955 (52.0% male, 48.0% female) were included. Most (80.97%) use prostate-specific antigen (PSA) and digital rectal examination (DRE) for screening; 9.4% use DRE alone and 7.15% PSA. Of the respondents, 8.3% do not offer prostate cancer screening. Most physicians begin offering screening at age 50 (72.9%) and stop at ages 70 or 80 (68.4%); 17.9% offer lifelong screening. In response to the statement "screening with DRE provides a survival benefit," 37.6% and 32.6% agreed and disagreed, respectively. For "screening with PSA provides a survival benefit," 43.3% agreed and 31.0% disagreed. For the statement "the benefits of prostate cancer screening outweigh the risks," 51.4% agreed and 22.0% disagreed. : Although 91.7% of respondents offer prostate cancer screening, they are divided over its utility. Only 51.4% were convinced that the benefits outweighed the harms. There is significant variability between physicians' screening protocols. A limitation of this study is the possibility of selection bias. Nevertheless, this is the largest sample of Ontario family physicians ever surveyed about prostate cancer screening and highlights divergent physician practices and a need for more conclusive evidence on screening utility.

Prostate cancer screening in the workplace. Employer costs.

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Recognition of the mortality and morbidity associated with prostate cancer has resulted in employer based screening programs. This retrospective cohort study identified the employer costs of prostate cancer screening and referrals due to abnormal test results. The subjects were 385 men enrolled in a workplace screening program at a single employer between 1993 and 1995. Screening consisted of digital rectal examination (DRE) annually for enrolled employees aged 40 years and older, plus annual prostate specific antigen (PSA) testing for those 50 and older, and those 40 and older and considered at high risk. Data related to the health care and lost productivity costs of screening and referrals for abnormal test results were collected and analyzed. The total cost of screening was \$44,355, or approximately \$56 per screening encounter (788 DREs; 437 PSAs). Abnormal screening tests resulted in 52 referrals. Upon further evaluation, 42% were found to have an enlargement, 29% a node, and 12% benign prostatic hyperplasia. Only one malignancy was found. The total cost of additional referrals was \$31,815, or 42% of the cost of screening plus referrals. As the cost per screening encounter was low, prostate cancer screening in the workplace is an efficient alternative.

American Cancer Society guideline for the early detection of prostate cancer: update 2010.

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In 2009, the American Cancer Society (ACS) Prostate Cancer Advisory Committee began the process of a complete update of recommendations for early prostate cancer detection. A series of systematic evidence reviews was conducted focusing on evidence related to the early detection of prostate cancer, test performance, harms of therapy for localized prostate cancer, and shared and informed decision making in prostate cancer screening. The results of the systematic reviews were evaluated by the ACS Prostate Cancer Advisory Committee, and deliberations about the evidence occurred at committee meetings and during conference calls. On the basis of the evidence and a consensus process, the Prostate Cancer Advisory Committee developed the guideline, and a writing committee drafted a guideline document that was circulated to the entire committee for review and revision. The document was then circulated to peer reviewers for feedback, and finally to the ACS Mission Outcomes Committee and the ACS Board of Directors for approval. The ACS recommends that asymptomatic men who have at least a 10-year life expectancy have an opportunity to make an informed decision with their health care provider about screening for prostate cancer after they receive information about the uncertainties, risks, and potential benefits associated with prostate cancer screening. Prostate cancer screening should not occur without an informed decision-making process. Men at average risk should receive this information beginning at age 50 years. Men in higher risk groups should receive this information before age 50 years. Men should either receive this information directly from their health care providers or be referred to reliable and culturally appropriate sources. Patient decision aids are helpful in preparing men to make a decision whether to be tested.

With increasing trends of prostate cancer in the Saudi Arabia and Arab World: Should we start screening programs?

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Incidence rate for prostate cancer in the Arab World is significantly lower than United States and Europe, it ranges from 5.5% to 39.2%. However, the incidence and the number of deaths is expected to increase. In Saudi Arabia, the crude incidence rate and age standardized incidence rate of prostate cancer are reported to be steadily increasing in between 2001-2008. Only two screening trials were attempted in 2001 and 2009 which yielded an incidence rate of 1.17% and 2.5% respectively. Men in the Arab world are sharing a common characteristic of poor knowledge and poor attitude towards prostate cancer examination and screening practices. They are ill-informed about the PSA test's strengths and drawbacks because the doctors are not talking to them about the importance of counselling. Men should be encouraged to do PSA testing before the age of 50 and till the age of 70 years. This could be achieved by enhancing their attitude and enriching the knowledge of the physicians towards PSA testing, harms and benefits, through shared decision making, which would increase men's knowledge scores, reduced their decisional conflict and promote greater involvement in decision making.

The Adult Well-Male Examination.

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The adult well-male examination should provide evidence-based guidance toward the promotion of optimal health and well-being. The medical history should focus on tobacco and alcohol use, risk of human immunodeficiency virus and other sexually transmitted infections, and diet and exercise habits. The physical examination should include blood pressure screening, and height and weight measurements to calculate body mass index. Lipid screening is performed in men 40 to 75 years of age; there is insufficient evidence for screening younger men. One-time screening ultrasonography for detection of abdominal aortic aneurysm is recommended in men 65 to 75 years of age who have ever smoked. Screening for prostate cancer using prostate-specific antigen testing in men 55 to 69 years of age should be individualized using shared decision making. Screening for colorectal cancer should begin at 50 years of age for average-risk men and continue until at least 75 years of age. Screening options include fecal immunochemical testing, colonoscopy, or computed tomography colonography. Lung cancer screening using low-dose computed tomography is recommended in men 55 to 80 years of age who have at least a 30-pack-year smoking history and currently smoke or have quit within the past 15 years. Immunizations should be updated according to guidelines from the Advisory Committee on Immunization Practices.
